



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : علياء عمر ابو الفتوح محمد
المحافظة : الجيزة
العمر (تاريخ الميلاد) : 21-05-1991 المهنة : بدون عمل
رقم البطاقة : 29105210101948 رقم التليفون : 01063405272
العنوان : ش الزعفراني التبين
البلد التبين القاهرة
تاريخ تسجيل الإستمارة : 03-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندي حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المدونة في هذا النموذج صحيحة.
رئيس اللجنة ومدير المستشفى.
تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----
قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: علياء عمر ابو الفتوح محمد National ID: 29105210101948	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



علاء
عمر ابو الفتوح محمد
قرية الشويك الشرقى
مركز الصف - الجيزة

١٩٩١/٠٩/٢١ ٢٩١.٥٢١.٠١.١٩٤٨
JW3835345

٢٠٢٢/٠٢ ٢٩١.٥٢١.٠١.١٩٤٨



بدون عمل
أنثى
مسلمة
متزوجة
راند عاطف احمد حنفى العلى
البطاقة سارية حتى ٢٠٣٠/٠٢/٢٤



١



Patient's Name: Alaia Omar Abo Al-Fotouh Mohammed
Date: 2026-07-11
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA.

As compared to previous study dated 11/4/2025, the current study reveled,

I- CHEST:

- No appreciable changes as regarding the multiple scattered pulmonary nodules and masses largest measuring 43 x 36 compared to 41x39mm at the previous study.
- Stationary course of the multiple mediastinal lymph nodes largest measuring 29 x 19 mm at the pretracheal region.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No appreciable changes as regrading the multiple bilobar hypodense focal lesions largest measuring 15 x 14 mm at segment IV as compared to 13x11mm at the previous study. No dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Still seen multiple axial and appendicular skeleton lytic bone lesions with D8 mixed lytic& sclerotic lesion with evidence of D5 vertebral body structural collapse.

Opinion:

❖ Overall stationary course

Dr.sherif Wagdy

Name : Alaia Omar Abu Al-Fotouh

Date : 22-06-2026 07:41

Mohammed

Branch : Dokki

Medical No. : 399472

Ref.Physician Prof : Goustav Roussey Egypt

SKELETAL SCINTIGRAPHY REPORT

34-year-old female with left breast carcinoma. Status post intramedullary nail/screw fixation of distal left femur. Follow-up for skeletal disease progression.

Technique: Whole-body planar bone scintigraphy (anterior/posterior) 2-3 hours post-IV Tc-99m MDP.

COMPARATIVE FINDINGS (vs. 09/03/2026):

Progressive osteoblastic metastatic disease:

- Skull: Pre-existing lesions increased, now large confluent dense uptake.
- Axial skeleton: Sternal and cervicodorsolumbar vertebral foci show increased intensity and spatial extent.
- Ribs: Right 7th and 9th lateral aspects—stable to mildly progressive.
- Pelvis: Left pubic bone and left SI joint—previously subtle, now intensely active with widened boundaries.
- Femurs: Proximal right femoral shaft—subtle focus now dense, highly active. Left femur—persistent diffuse intense uptake throughout shaft/distal metaphysis, reflecting active disease plus reactive bone remodeling around hardware.

IMPRESSION

- Progressive metastatic disease compared to 09/03/2026, with concurrent increased intensity and spatial expansion—confidently excluding flare phenomenon.
- Left pelvic and right femoral lesions, previously subtle, have evolved into confluent active osteoblastic metastases.
- Left femur disease is shielded by hardware; however, the rapidly advancing right proximal femoral shaft lesion involves an unprotected weight-bearing cortex, posing immediate pathological fracture risk.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية



04/27/2026

MDT FORM

Name:	Alyaa Omar Abo Elfoutoh Mohamed	MRN:	1841
Age:	35 Y	ID:	29105210101948
Menopausal Sta	pre	Phone:	01063405272
NRS SCORE:	3	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX:

ER: PR: HER2: KI67:

Sonomammography:

Mets Workup:

History:

prem mbc to liver lung bone er 8/8 pr4/8 her23+ history left breast cancer rec nact left mrm 9/2022 ypt1 ypn0 rec kadcyla last 6/2023 7/2023 mets to bone st xeloda tykerb then navelbine tykerb 2/2024 dp herceptin gemzar then dp newly dev hfl st taxotere x5 dp rec carbopl x3 dp in pul nodsules pt rec oonly 7 weeks taxol and 3 herceptin cts 4/2026 no comparison done bilat pul nodules med in m 27x26 mm (last cts 2 no med in) bilobar hfl largest seg 7 m 13x13 mm (last ct2/2026 bilobar hfl largest seg 5 12x11mm) for comparison of cts for mdt meanwhile continue taxol for w8 taxol

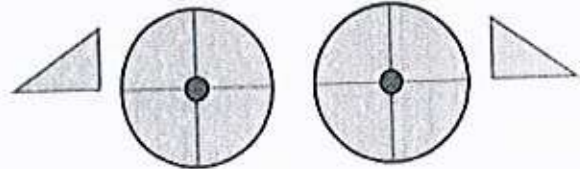
Recommendations:

MDT Outcome & Plan

No frank DP : to continue taxol-herceptin for another 2 months then re-evaluate by properly compared CT CAP

R

L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ نوره مصلح	د/ نوره مصلح				

Patient Name : الأستاذ / علياء عمر ابو الفتوح	Request No. : 13005214
Patient No. : 29105210101948	Request Date : 26/02/2026
Path No. : 556/26	Verification Date : 07/03/2026
Sex : Female	Financial Category : جوستاف روسي - نفقة الدولة
Age : 35 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Multiple hepatic focal lesions in a patient with history of breast cancer.

Nature of Specimen: Hepatic lobe focal lesion, Tru-cut biopsy, collected on 26/02/2026, at 2:20 pm, cold ischemia time: 20 seconds.

Gross Examination:

- Two grey white tissue cores, each measured 1.5 cm long. Totally submitted.

Microscopic Examination:

* Histopathological examination reveals cores of hepatic tissue showing infiltration by tumor formed of malignant epithelial cells showing moderate nuclear pleomorphism and eosinophilic cytoplasm, arranged in clusters with moderate tendency to tubular formation amid desmoplastic stroma.

Diagnosis:

Hepatic lobe focal lesion, Tru-cut biopsy:

Metastatic carcinomatous deposits consistent with breast origin.

- Immunophenotyping is in a separate report.

Signature

Prof. Dr. Maha Guimei.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 07/03/2026 10:12:05

Page 1 of 1

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذ / علاء عمر أبو الفتوح	Request No.	: 13005214
Patient No.	: 29105210101948	Request Date	: 26/02/2026
Path No.	: 556/26	Verification Date	: 07/03/2026
Sex	: Female	Financial Category	: جوسف روسي - نفقة الدولة
Age	: 35 Y	Department	: Out Patient
Referreing Doc. :		IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Multiple hepatic focal lesions in a patient with history of breast cancer.

Nature of Specimen: Hepatic lobe focal lesion, Tru-cut biopsy, collected on 26/02/2026, at 2:20 pm, cold ischemia time: 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER clone (1E2), PR clone (SP1), Her2 clone (4B5), GATA3 (L50-823).

Conclusion:

- * IPT (All controls showed appropriate reactivity):
- GATA3: Diffuse positive staining in tumor cells.
- ER: Strong nuclear staining in 90% of tumor cells (score 8/8).
- PR: Negative nuclear staining in tumor cells (score 0/8).
- Her2: Strong complete membrane staining in more than 10% of tumor cells (score 3+).

Results:

Hepatic lobe focal lesion, Tru-cut biopsy:

Metastatic carcinomatous deposits consistent with breast origin.

IPT:

- GATA3: Positive in tumor cells.
- ER: Positive (score 8/8).
- PR: Negative (score 0/8).
- Her2: Positive (score 3+).

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%)

Verified by :

Dr.Duaa Mohamed

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Page 1 of 2

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Patient Name الأستاذ / علياء عمر أبو الفتوح
Patient No. 29105210101948
Path No. 556/26
Sex Female
Age 35 Y
Referring Doc.

Request No. 13005214
Request Date 26/02/2026
Verification Date 07/03/2026
Financial Category جوستاف روسي - نفقة الدولة
Department Out Patient

Registered by Mrs. Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0 (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Prof. Dr. Maha Guimei.

Dr. Heba El-Shafeey.

Verified by :

Dr. Duaa Mohamed

Printing Date : 07/03/2026 10:12:10

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atient Name : الأستاذة / علياء عمر أبو الفتوح
atient No. : 29105210101948
ith No. : 2628/22
ex : Female
ge : 31 Y
efferreing Doc :

Request No. : 13001062
Request Date : 06/09/2022
Verification Date : 20/09/2022
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : Ali mostafa

Pathology Report

Clinical Data:

Left breast cancer, post NACT.

Preparation of the specimen:

Left sparing mastectomy, Axillary lymph nodes.

Gross Examination: Three specimens were received:

- Base of nipple: Fibrofatty tissue piece measured 2x1 cm, for frozen section, no gross duration.
- Skin sparing mastectomy: Fibrofatty tissue piece measured 20x15x3 cm. Cut section revealed three grayish white firm tumour bed site measured 1.2x1x1cm, 1x1x1cm and 0.5x1x1cm.

Axillary dissection revealed 18 axillary lymph nodes up to 1x1cm.

- Separate left axillary lymph nodes: fibrofatty tissue piece measured 2x2 cm, entangling 2 nodules up to 0.3x0.3cm.

Microscopic examination:

Histological type: Multifocal Invasive duct carcinoma.

Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+1) Grade II

Skin involvement: Free nipple.

DCIS: Yes.

Angioinvasion: Negative

Lymph node involvement: Free axillary lymph nodes (0/21)

Resection Margins: Free

Histologic response to therapy: No pathological response.

Frozen section: Base of nipple free of tumor cells.

Diagnosis:

Left breast cancer; Left nipple sparing mastectomy and Axillary dissection:

Verified by :

DR.Menna Nabil

Printing Date : 20/09/2022 13:23:38

Page 1 of 2

Printing By : DR.Menna Nabil

ient Name : الأستاذ / علياء عمر ابو الفتوح	Request No. : 13001062
ient No. : 29105210101948	Request Date : 06/09/2022
1 No. : 2628/22	Verification Date : 20/09/2022
2 : Female	Financial Category : نفقة الدولة - اورام دار السلام
3 : 31 Y	Department : Out Patient
Referring Doc :	Registered by : Ali mostafa

Pathology Report

Multifocal Invasive duct carcinoma grade II with focal Ductal carcinoma insitu.

Free margins.

Free axillary lymph nodes (0/21)

No pathological response to therapy.

Stage: ypT1b, ypN0

Prof. Dr. Gamal Abdelaty.

Prof. Dr. Amal Hareedy.

Dr. Samah Sayed.

Verified by :

DR.Menna Nabil

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Page 2 of 2

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Room No. (4) - First Floor-Building No. 557- Nile Corniche - Nile El Qadima- Cairo, Egypt

Patient Name : الأستاذ / علياء عمر أبو الفتوح	Request No. : 139122
Patient No. : 29105210101948	Request Date : 16/01/2022
Path No. : 165/2022	Verification Date : 06/02/2022
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 31 Y	Department : Out Patient
Referring Doc. :	IHC No. :
	Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass

Nature of the specimen: Tru-cut biopsy

Gross Examination: Multiple greyish white and tan tissue cores up to 1.5 cm, totally submitted.
Procedure: Tru-cut biopsy

Microscopic examination:

Histological type: Invasive duct carcinoma

Grade (differentiation+ nuclear pleomorphism + mitosis): GII (2+2+2)

Angiolymphatic invasion: N/A

Ductal carcinoma in situ: N/A

Diagnosis:

Left breast mass; Tru-cut biopsy:

Invasive duct carcinoma GII

Immunohistochemistry:

HER-2: Strong nuclear reaction in about 95% of tumour cells. Positive (Allred score 8/8)

HER-1: Moderate nuclear reaction in about 40% of tumour cells. Positive (Allred score 4/8)

HER-2: Strong complete membrane reaction in 10% of tumour cells. Positive, score 3+

HER-3: 40%

Prof. Dr. Amal Hareedy

Dr. Menna Atef

Dr. Walaa Ghonaim

Dr. Abdulrahman Ibrahim

Verified by :

abdelrahman ibrahim

Printing Date : 06/02/2022 13:40:39

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